

OPERATIVE REPORT

SURGEON: Sarah Spine, MD

ANESTHESIOLOGIST: Victor Vapor, MD (Independent Group #44)

PROCEDURE DATE: May 12, 2026

PREOPERATIVE DIAGNOSIS:

L4-L5 Disc Herniation with Left-Sided Radiculopathy (M54.16).

PROCEDURE PERFORMED:

1. Lumbar Microdiscectomy, L4-L5 Left.
2. Use of Operating Microscope for Microsurgical Dissection.

OPERATIVE SUMMARY:

After induction of general anesthesia, the patient was placed in the prone position on the Andrews frame. A 3cm midline incision was made over the L4-L5 interspace. Using the operating microscope, a laminotomy was performed. The thecal sac was retracted medially to reveal a **large, extruded disc fragment** impinging heavily on the L5 nerve root. The fragment was removed in one piece. The nerve root was then noted to be free and decompressed. Hemostasis was achieved. Closure was performed in multiple layers.

ESTIMATED BLOOD LOSS:

Minimal (< 50cc). No complications encountered.

